

75. Global Considerations in Kidney Disease Africa · assets 1-9 / 13

Fig. 75.1 World population estimates by region, with projections to 2050. (From World Population Prospects 2022. Summary of results. United Nations, Department of Economic and Social Affairs, Population Division; 2022.)

Fig 75 1

Fig. 75.4 Prevalence of nephrologists in African countries, per million population. (From Bello AK et al., 2023, ISN-Global Kidney Health Atlas, ISN, Brussels. Available at www.theisn.org/global-atlas.) Map lines delineate study areas and do not necessarily depict accepted national boundaries.

Fig 75 4

Fig. 7.57 The spectrum of kidney disease in low- and middle-income countries (LMICs). Acute kidney injury (AKI), acute kidney disease (AKD), and chronic kidney disease (CKD) are interconnected syndromes. AKD and CKD can occur from the progression of AKI but also increase susceptibility to repeated episodes of AKI. Populations in LMICs may be more susceptible to kidney disease through genetic risk factors, lower nephron endowment, and chronic exposures or repeated episodes of kidney injury across the lifespan. (Adapted with permission from Kalyesubula R, Contoy AL, Galce-Silva V, et al. Screening for kidney disease in low- and middle-income countries. *Semin Nephrol*. 2022;15:1315.)

Fig_75_7

Fig. 75.2 Age distribution of the African population, by subregion. (From Africa Overview. 2022 World Population Data Sheet. Population Reference Bureau: 2022.)

Fig 75 2

Fig. 75.5 Etiologic factors for acute kidney injury in Africa. (From Mehta RL, Macedo E, Zhang J, ISN Oby25 Initiative Steering Committee. International Society of Nephrology Oby25 Global Snapshot. Regional report for Africa; 2015.)

Fig 75 5

Fig. 75.5 Proposed screening algorithm for chronic kidney disease (CKD) in low- and -middle-income countries (LMICs). AKI, Acute kidney injury; GFR, glomerular filtration rate; HIV, human immunodeficiency virus; UACR, urine albumin-to-creatinine ratio. (Adapted with permission from Kalyesubula R, Conroy AL, Calico-Silva V, et al. Screening for kidney disease in low- and middle-income countries. *Semin Nephrol*. 2022;151315.)

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Fig. 75.3 Total expenditure on health as percentage of gross domestic product in the Africa region, 2020. (From The World Bank, Health spending as percent of GDP—country rankings. *TheGlobalEconomy.com*, 2020. <https://www.theglobaleconomy.com/rankings/health_spending_as_percent_of_gdp/#africa>; 2020. Accessed 13.11.23.)

Fig 75 3

Fig. 75.6 Prevalence of etiologic factors for acute kidney injury in adults and children in sub-Saharan Africa. (Adapted from Olowu WA, Niang A, Osafo C, et al. Outcomes of acute kidney injury in children and adults in sub-Saharan Africa: a systematic review. *Lancet Glob Health*. 2016;4:e242–250.)

Fig 75 6

Fig. 75.9 Differences between North Africa and sub-Saharan Africa in the prevalence of common glomerular diseases. *CresGN*, Crescentic glomerulonephritis; *DPGN*, diffuse proliferative glomerulonephritis; *FFGN*, focal proliferative glomerulonephritis; *FGSG*, focal segmental glomerulosclerosis; *Hep B*, hepatitis B virus-associated nephropathy; *HIVAN*, HIV-associated nephropathy; *IGN*, IgA nephropathy; *LN*, lupus nephritis; *MCD*, minimal change disease; *MCGN*, mesangiocapillary glomerulonephritis; *MesPGN*, mesangio proliferative glomerulonephritis; *MGN*, membranous glomerulonephritis; *PIGN*, postinfectious glomerulonephritis; *SSA*, sub-Saharan Africa.

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